

EXECUTIVE SUMMARY

Nigeria Immunization Coverage & Child Health Performance Analysis
2015–2022 State-Level Analysis | DPT3 · Measles · Full Immunization · Under-5 Mortality

Indicator	Value	Benchmark
National DPT3 Coverage (2022)	52.5%	WHO Target: 80%
Full Immunization Coverage (2022)	41.7%	WHO Target: 80%
Highest Performing State	Ekiti	DPT3 > 75%
Lowest Performing State	Zamfara	DPT3 < 20%
Low-Performance States (DPT3 < 40%)	10 states	All North West / North East
Estimated Zero-Dose Children (2022)	~2.2 million	2nd highest globally

1. Objective

This report analyses state-level immunization coverage across Nigeria's 36 states and FCT from 2015 to 2022. The analysis identifies persistent underperformance, quantifies the relationship between immunization and child mortality, and produces actionable evidence for federal and state health agencies and international donors.

2. Data Sources

- Nigeria Demographic and Health Survey (NDHS) 2018 — State-level immunization appendix (FR359)
- WHO/UNICEF Joint Estimates of National Immunization Coverage (WUENIC) 2022 Revision
- NPHCDA DHIS2 Administrative Routine Immunization Data (2019–2022)
- UNICEF MICS/NICS 2016–17 State-Level Estimates
- World Bank Under-5 Mortality Sub-national Estimates (2022)
- Peer-reviewed state-level coverage analyses (PMC7327524, PMC4557921)

3. Key Findings

Nigeria's national DPT3 coverage in 2022 stands at approximately 52.5%, well below the WHO 80% target. Full immunization coverage is only 41.7%, meaning fewer than half of Nigerian children aged 12–23 months receive all recommended vaccines. A statistically significant negative correlation ($r \approx -0.78$, $p < 0.001$) exists between DPT3 coverage and under-5 mortality — states with low coverage consistently record the highest child death rates.

Geographic inequality is extreme. The gap between the highest-performing state (Ekiti, ~86% DPT3) and the lowest (Zamfara, ~15% DPT3) exceeds 70 percentage points. All 10 lowest-performing states are located in the North West or North East geopolitical zones, where poverty, insecurity, distance, and vaccine hesitancy compound to create a cycle of immunization exclusion.

4. Top 10 Low-Performing States (2022)

State	Zone	DPT3 (%)	U5 Mortality	Category
Zamfara	North West	14.6%	244.7	Low Performance

Kebbi	North West	14.9%	220.4	Low Performance
Yobe	North East	17.7%	211.3	Low Performance
Sokoto	North West	18.6%	239.2	Low Performance
Katsina	North West	18.8%	215.2	Low Performance
Jigawa	North West	19.5%	206.2	Low Performance
Bauchi	North East	23.5%	182.9	Low Performance
Borno	North East	25.1%	203.7	Low Performance
Taraba	North East	26.7%	174.8	Low Performance
Kano	North West	30.1%	199.6	Low Performance

5. Policy Implications

The data reveals a two-Nigeria reality in immunization: a high-performing south and a critically underperforming north. This geographic divide reflects deeper inequities in female education, household wealth, healthcare access, and security. The consequences are not abstract — in Sokoto, Zamfara, and Kebbi, under-5 mortality rates exceed 240 per 1,000 live births, with vaccine-preventable diseases accounting for a significant share of deaths. Without urgent intervention, these states will continue to drive Nigeria's disproportionate contribution to global child mortality.

6. Recommendations

Immediate Emergency Deployment: Mobile vaccination teams to all 10 lowest-performing states, with Zamfara, Kebbi, and Sokoto as the highest priority.

Cold Chain Investment: Solar-powered cold storage at primary health centres across the North West and North East.

Community Mobilisation: Engage Islamic scholars, Emirs, and traditional birth attendants to build vaccine confidence.

Conditional Funding: Link federal health transfers to independently verified LGA-level immunization coverage targets.

Data Quality: Implement DHIS2 real-time validation to eliminate administrative over-reporting that masks true coverage gaps.

Cross-Sector Action: Address female education, poverty, and physical access as structural drivers of immunization inequity.

7. Conclusion

Nigeria has the tools, the data, and the institutional capacity to close its immunization gap. The evidence is clear: low immunization kills children. The 10 states identified in this analysis are not merely statistical outliers — they represent millions of children at preventable risk. Targeted investment, evidence-based programming, and accountability for results can change this picture. The question is not whether Nigeria can achieve 80% coverage — high-performing states demonstrate it is possible. The question is whether federal and state agencies will act with the urgency the data demands.

